

Knowledge Mobilization Gaps in the Communication of Ovarian Cancer Detection Without Population Screening

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The Gap Between Evidence and Understanding

- There is no population-based screening test anywhere.
- CA-125 (blood tumor marker test) and transvaginal ultrasound (an internal pelvic ultrasound) have not reduced deaths in routine screening.
- Pap test misconceptions are common.
- False reassurance can make people less likely to notice symptoms or seek care.

Women who believe the Pap test detects ovarian cancer

Source: scoping review findings

Lockwood-
Rayermann (2009)
Women 40+

67%*

Hawkins (2011)
Adult women, general public

40.6%

Cooper (2016)
Women, general public

31.1%

Cooper (2011)
Women 40–60, focus groups

Qualitative: common belief

* "More than two-thirds" reported. Cooper 2011 found the belief without a percentage. Data from scoping review findings.

Research Question & Project Aim

- Primary aim: identify knowledge mobilization gaps in how ovarian cancer detection is communicated when no population-based screening exists.
- Research question: **What evidence and knowledge mobilization resources address the communication of ovarian cancer detection in the absence of population-based screening?**
- Map how detection is communicated across public and clinical sources, then build tools that respond directly to where it is failing.
- Women with early symptoms, primary care providers, and community organizations benefit from clearer, evidence-based communication.

How This Project Was Conducted

METHOD

Scoping review

Arksey and O'Malley framework
with PRISMA-ScR reporting

Maps what exists, not one intervention

SOURCES SEARCHED

Databases: PubMed, CINAHL, Web of Science

Grey literature: National Institute for Health and
Care Excellence, American College of Obstetricians
and Gynecologists, Ovarian Cancer Canada

SCREENING

1,660

records identified



1,154

screened after duplicates



72

full texts assessed



30

studies included

FINDINGS INFORMED A FOUR-COMPONENT TOOLKIT

Patient
Fact Sheet

Clinician
Conversation Guide

Digital
Myth-Busting Page

Awareness
Measurement Tool

What the Evidence Tells Us

- The United Kingdom Collaborative Trial of Ovarian Cancer Screening (UKCTOCS) found that routine ovarian cancer screening did not reduce deaths after 16-year follow-up and caused harm through false positives.
- Public misunderstanding is widespread.
- The gap extends to providers. Studies show family physicians and nurse practitioners also held incorrect beliefs about CA-125 and screening.
- Symptom normalization and delayed help-seeking show why communication must happen earlier.

What This Means and What Comes Next

- The key opportunity is earlier than oncology: symptom recognition, care-seeking, and provider communication.
- The toolkit targets that earlier window: primary care, community health, and public-facing spaces, where communication can still change outcomes.
- Each component answers a documented gap:
 - The Fact Sheet and Myth-Busting Page reduce false reassurance.
 - The Conversation Guide supports clear clinical messaging.
 - The Measurement Tool enables evaluation.
- Next steps: pilot with patients and primary care providers, integrate into community settings, and study which approaches most reduce diagnostic delay.